

ST. MARY'S HOSPITAL DISCHARGE SUMMARY

Patient: Diane Morgan
DOB: 22.05.1961
Admission: 13.02.2024
Discharge: 16.02.2024
MRN: AML060

ADMISSION DIAGNOSIS: Acute myeloid leukemia post alloTx with central line infection.

DISCHARGE DIAGNOSIS:

1. AML - NPM1/FLT3-ITD positive, intermediate risk, in sustained complete remission, post alloTx May 2022
2. Central line-associated bloodstream infection (CLABSI) - Staphylococcus epidermidis, resolved
3. Hypertension
4. Hypothyroidism

HOSPITAL COURSE:

Mrs. Morgan is a 62-year-old female diagnosed with AML on January 21nd, 2022. Initial presentation included fatigue, petechiae, and pancytopenia. Bone marrow revealed 78% blasts with normal karyotype, NPM1 mutation (VAF 47%), and FLT3-ITD (allelic ratio 0.65), classifying as intermediate-risk disease.

Given her age and fitness level (ECOG 0), she was considered for intensive therapy but elected hypomethylating agent approach. Treatment initiated February 7th, 2022, with venetoclax 400mg daily days 1-28 plus azacitidine 75mg/m² days 1-7 of 28-day cycles.

She achieved complete remission after cycle 1, confirmed by bone marrow biopsy showing <5% blasts, normal flow cytometry, and molecular clearance of NPM1 mutation. FLT3-ITD became undetectable after cycle 3. We proceeded with allogeneic stem cell transplantation in May 2022. She developed grade 1 skin GvHD, which was controlled with a steroid course. Since then, she has remained in complete remission.

Current admission triggered by fever (38.7°C) and positive blood cultures growing Staphylococcus epidermidis from her port. She was started on vancomycin with rapid clinical improvement. Repeat cultures negative. Port was salvaged successfully.

DISCHARGE MEDICATIONS:

1. Venetoclax 400mg PO daily days 1-28
2. Azacitidine 200mg subcutaneous days 1-7
3. Allopurinol 300mg daily
4. Acyclovir 400mg BID
5. Lisinopril 10mg daily
6. Levothyroxine 88mcg daily

7. Ondansetron 8mg q8h PRN nausea

FOLLOW-UP: Bone marrow reassessment in 3 months.

LABORATORY DATA AT DISCHARGE:

WBC 4.8 (ANC 2.9), Hgb 12.1, Platelets 185

Basic metabolic panel within normal limits

LFTs normal

Dr. Patricia Williams, MD

Attending Hematologist